WHERE INDIA STANDS ON HEALTH

To achieve its universal health coverage goal, India must raise diagnostic capacities, improve the health worker-to-patient ratio, reduce out-of-pocket expenditure, expand insurance and implement price controls

DOCTOR-PATIENT RATIO

Allopathic doctors per 10,000 population

INDIA	7.265
USA	36.082
UK	31.742
RUSSIA	38.271
CHINA	25.183
AUSTRALIA	39.812

Source: WHO

HOSPITAL BED-PATIENT RATIO

Beds per 10,000 population

INDIA	16
USA	27.4
UK	23.5
RUSSIA	70.3
CHINA	50
AUSTRALIA	38.4

Source: WHO

OUT-OF-POCKET EXPENDITURE

Share in current health expenditure

INDIA	47.1%
USA	10.7%
UK	13.5%
RUSSIA	27.22%
CHINA	34.39%
AUSTRALIA	13.82%

Source: World Bank

PUBLIC HEALTH EXPENDITURE

Share in current GDP

INDIA	2.1%
USA	16.6%
UK	11.3%
RUSSIA	5.7%
CHINA	10%
AUSTRALIA	11.5%

Source: Statista

MORTALITY RATE OF NONCOMMUNICABLE DISEASES

Per 100,000 population

INDIA	558.5
USA	407.9
UK	329.9
RUSSIA	491.5
CHINA	278.5
AUSTRALIA	235.4

Source: WHO

MATERNAL MORTALITY RATIO

Per 100,000 live births

INDIA	103
USA	21.1
UK	9.8
RUSSIA	13.7
CHINA	23
AUSTRALIA	4.31

Source: WHO

INFANT MORTALITY RATE

Between birth and 11 months per 1,000 live births

INDIA	25.5
USA	5.4
UK	3.6
RUSSIA	3.8
CHINA	4.8
AUSTRALIA	3.2

Source: WHO

NEONATAL MORTALITY RATE

Between 0 and 27 days per 1,000 live births

INDIA	18.1
USA	3.2
UK	2.7
RUSSIA	1.8
CHINA	3
AUSTRALIA	2.3

Source: WHO

INDIA USA UK RUSSIA CHINA AUSTRALIA JAPAN

GLOBAL API* PRODUCTION

Over 500 different APIs are made in India

**Active Pharmaceutical Ingredient*



India **7%**
USA **41%**
China **18%**
Other nations **34%**



India **20%**
China **44%**
Other nations **36%**

Source: Market Wrap by ICICI, 2019

32% of India's API needs are still met through imports

already has Truenat machines and GeneXpert tests, but one needs significant infrastructure and manpower to run them, which isn't easy to deploy at the lower levels. We are looking at point-of-care machines to enable diagnosis at levels where none is happening currently," says Sanjay Sarin, country director, FIND India, which is partnering with various states to build diagnostical capacities. A vast number of patients are compelled by circumstances to rely on facilities at the local level, and "it takes a significant amount of time for samples to be sent to the higher levels. One would want to create a sort of multiplex that enables running multiple tests on the same platform, and not just for TB diagnosis. The government is also considering handheld X-ray units which can be taken into the community for diagnosis."

However, scaling up diagnostics can only be done if the number of frontline workers who will take them to commu-